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U.S. Department of Health and Human Services
330 C Street SW, 7th Floor
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Submitted via Regulations.gov

RE: **Request for Information; Health Technology Ecosystem [CMS-0042-NC]**

Dear Ms. Carlton and Mr. Posnack:

On behalf of Ochsner Health (Ochsner), thank you for the opportunity to provide input in response to the request for information (RFI) on the "Health Technology Ecosystem" [CMS-0042-NC]. We commend the Centers for Medicare and Medicaid Services (CMS) and the Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology (ASTP/ONC) for collaborating on this solicitation of feedback on health technology infrastructure and inviting public input regarding "effective digital capabilities needed to make informed health decisions." Ochsner has a proven track record of deploying digital health tools, including wearables, which have enhanced the patient experience, improved chronic disease management and clinical outcomes, and reduced costs; our lessons learned, insights, and recommendations are below.

About Ochsner Health

As one of the nation's leading non-profit, academic health systems, Ochsner is dedicated to delivering accessible, affordable, convenient, and effective person-centered care to the more than one million patients we serve who come from every state in the nation and 62 countries across the world. Headquartered in New Orleans, Ochsner consistently is named both the top hospital and top children's hospital in Louisiana by U.S. News & World Report. Ochsner is one of the leading health systems in the country for cancer care, cardiology, neurosciences, liver and heart transplants, and pediatrics. An independent academic medical center, Ochsner performs groundbreaking clinical research on new treatments and emerging medical technologies in nearly 700 active clinical trials.

Ochsner delivers a comprehensive range of services throughout Louisiana, Mississippi, and the Gulf South region with a clinically integrated network of 48 owned, managed, and affiliated hospitals, and more than 370 health centers and urgent care clinics. Ochsner offers a wide variety of specialized and nationally ranked services with its more than 4,700 affiliated physicians, including 1,800 employed physicians practicing in more than 90 specialties and subspecialties and more than 38,000 employees.

Ochsner's Experience & Perspective on Digital Health Tools

1. Digital Health Apps

Question PR-1. *What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC-5) digital health products for their patients?*
a. What are the current obstacles?
b. What information should providers share with patients when using digital products in the provision of their care?
c. What responsibilities do providers have when recommending use of a digital product by a patient?

Response to PR-1

(a) Ochsner has taken several concrete steps to expand the availability of digital health products for patients. In response to the challenge of improving care and outcomes while reducing costs, in 2015 Ochsner created an innovation lab, innovationOchsner (iO), to improve health through innovation. iO has developed numerous digital medicine programs that are transforming the patient experience, enhancing health, and well-being, while decreasing costs and reducing the stress on providers of care. Critically, our pioneering telehealth and digital health programs are meaningfully increasing patient access to care for people in rural and other underserved areas of Louisiana and Mississippi where, in certain cases, no such access existed before.

In our experience, the primary obstacle to leveraging digital health tools is coverage and payment requirements that are inflexible and reimbursement rates and constructs that do not keep pace with technology uptake. In particular, Medicare and Medicaid mostly fail to reimburse the cost of digital health devices and the associated services, despite pilot programs demonstrating savings overall.

- For example, in 2020, Ochsner Digital Medicine (ODM) launched a Louisiana Medicaid demonstration project to provide remote patient monitoring and management (RPM)[1] to patients with chronic disease, with the goal of improving health outcomes for patients and lowering the cost of care for Louisiana Medicaid.

- In the demonstration, patients used a digital device to submit physiological readings to a dedicated care team of providers, health coaches, and pharmacists who were monitoring the submissions, following-up with patients to make adjustments to care, and providing ongoing

[1] Ochsner's digital health programs are beyond just "monitoring" as they provide active disease management and engagement with patients; as such, Ochsner does not use the term "remote patient monitoring" but prefers "remote patient management." For the purposes of this submission, we use RPM to mean a digital health program that involves a patient using a connected device to send physiological data to Ochsner health professionals and Ochsner health professionals regularly assessing the data provided and giving real-time feedback to the patient regarding any necessary adjustments to medications, nutrition, activity level, etc.

disease management. Providers leveraged a suite of behavioral, lifestyle change, and medication management tools to support patients via this RPM program.

- The pilot included, at various times, a total of 6,000 Medicaid patients and focused on individuals with hypertension and Type 2 diabetes. There was no added cost to the patient. The results were clear: emergency room visits, hospital admissions, and inpatient days all decreased, with a 4:1 return on investment for hypertension patients and a 3:1 return for diabetes. Across both programs, the total return on investment (in terms of estimated claims reductions minus costs) was $6.1 million.

Medicare and Medicaid could unlock these proven savings if the programs paid 100% for the upfront costs of providing the digital health tools and the ongoing monitoring rather than charging cost-sharing to patients. Currently, beneficiaries are required to pay cost-sharing (usually 20% or a fixed dollar copayment) for the digital health devices (e.g., Bluetooth enabled blood pressure cuffs) as well as pay cost-sharing for the ongoing RPM services and telehealth care. We know from our own patient survey data that these expenses often stand in the way of patient utilization.

- For Ochsner's Medicare beneficiaries who do not have supplemental coverage to cover out-of-pocket cost-sharing, we have found many decline the opportunity to participate in our digital medicine programs when they learn they would have to pay coinsurance. According to the CMS Current Beneficiary Survey (MCBS), in 2022, only 42% of beneficiaries with Medicare fee-for-service coverage had Medicare supplemental insurance to help cover cost-sharing obligations.[2]

- During the COVID-19 federally-declared Public Health Emergency (PHE), when we were permitted to waive beneficiary cost-sharing and due to these flexibilities provided by CMS, we experienced a significant increase in enrollment and participation among patients who need these digital health programs, which in turn helped improved their health and reduced costs.

    - Yet, following the end of the PHE providers were required to resume charging cost-sharing for digital health programs. Unfortunately, when we issued our annual reconsent to treat, which includes a cost notification of potential out-of-pocket expenses, **43% of beneficiaries who had been participating in the programs during the PHE** decided to opt-out of the program(s) when they learned that they could face 20% cost-sharing requirements. This illustrates the direct and significant adverse impact that cost-sharing has on participation in these important chronic disease management programs.

Further, CMS should simplify the documentation requirements for coverage and reimbursement for digital health programs and RPM and provide adequate reimbursement for the role that physicians, nurses, and other clinicians play in monitoring and evaluating the submitted physiological data and managing patients' chronic diseases via these tools and technologies.

Many beneficiaries have more than one chronic condition that should be monitored via RPM – such as hypertension and diabetes; yet CMS does not permit a provider to bill separately for two different chronic disease management programs at the same time even though resources are

[2] https://www.cms.gov/data-research/research/medicare-current-beneficiary-survey

needed to support each individual disease management RPM program. Currently, when multiple medical devices for different disease management purposes are provided to a patient, CMS only allows the services associated with all of the medical devices (e.g., blood pressure cuff and glucometer) and disease management programs to be billed once per patient per month and only when a minimum of 16 days of data have been collected. This means that in any given month, a Medicare beneficiary with both diabetes and hypertension generally cannot participate in more than one RPM program unless they find a provider who is able to offer them with two chronic disease programs – in essence for the cost of one – which is neither realistic nor feasible.

- Understanding that CMS may have concerns about fraud, waste, and abuse of RPM programs, Ochsner respectfully suggests that CMS provide greater flexibility to participants in value-based programs because the built-in incentives protect against over- or mis- use. Either through the Medicare Shared Savings Program (MSSP) or using the waiver authority granted to the Centers for Medicare and Medicaid Innovative (CMMI), CMS could: waive cost-sharing for beneficiaries; allow beneficiaries to participate in as many RPM disease management programs as their diagnoses and health care professionals determine is appropriate; and provide payment to providers for multiple RPM programs per patient and reduce the data collection requirements. These steps would improve chronic disease management and bend the chronic disease cost curve. *See further below under F. Value-Based Care Organizations (page 5) for more on this concept.*

Another example of how Ochsner is leveraging digital tools is our Virtual Nursing Program, where a team of virtual nurses complements the work of inpatient hospital bedside teams, using two-way audio and visual technology to allow for 24/7 communication. Virtual nurses can fulfill the administrative and documentation needs for multiple patients at a time and can coordinate care via real-time updates, freeing up the bedside care team to focus more on direct patient care. The Virtual Nurse Program has measurably reduced readmission rates, resulted in earlier detection of sudden changes in patient condition, optimized bed placement and capacity, protected critical care bed capacity for highly acute critical care patients, contributed significantly to nursing workforce retention, and resulted in higher Hospital Consumer Assessment of Healthcare Providers and Systems (CAHPS) scores.

Other programs at Ochsner have also used this type of virtual model, like the maternal monitoring TeleStork program and the eICU program. By utilizing a central “bunker,” where clinicians and/or nurses receive 24/7 live patient updates, Ochsner can provide faster, better responses throughout the medical system, including in rural areas where specialists and nurses may be in short supply. In other words, the “bunker” is focal point of a hub-and-spoke model, where monitoring and specialist care is consolidated in one central location and can coordinate in-person care at a series of individual facilities (the spokes).

- While these types of virtual programs have tremendous potential to improve care and outcomes for patients, tight inpatient margins mean that hospitals do not have sufficient resources to purchase and install the technology necessary to scale such efforts. CMS could consider providing a special payment as a one-time incentive to hospitals to develop and deploy such programs, which improve patient care, support the health care workforce, and increase access to care, particularly for rural communities.

(b, c) Regarding responsibilities vis-à-vis patients on recommending devices and sharing information with them, Ochsner believes transparency is paramount. For instance, Ochsner has developed a set of principles on the use of generative artificial intelligence (GenAI). GenAI is a rapidly evolving technology that is increasingly built into digital health products, with numerous serious issues to consider on capturing and processing sensitive patient data. Ochsner believes patients should know:

- Data privacy and security protections are in place.
- Where and how providers are deploying AI.
- Health care providers utilizing and/or relying on AI have received proper training.
- GenAI tools have been validated and certificated with any errors mitigated.
- The legal responsibilities of hospitals, health systems, health care providers, individual health care professionals, and AI developers.

Ochsner is actively following these principles in a variety of applications, including:

- ***Patient messaging*** – The MyOchsner App is a full-service digital health care portal that allows patients to access their medical records, schedule appointments, meet physicians and providers, view test results, and reach the 24/7 on-call nurse. Patients also commonly use the app to message their providers when they have a non-urgent health care need.

- ***Radiology –*** Ochsner has partnered with an outside company to deploy AI models in radiology that improve the radiologist workflow. The first deployment is to interpret CT scans performed for patients with suspected stroke in the emergency department. When the AI model detects a suspected stroke, there is an automatic notification to the radiologist to prioritize reading that study first, with a simultaneous notification to the emergency department to be on alert. Early results suggested a reduction in time to treatment of stroke by more than 30 minutes**.**

- ***Ambient Listening*** – New large language models can, with patient consent, "listen in" to a patient-clinician encounter and summarize it into clinicians' notes, reducing administrative burdens and improving the patient experience since physicians are no longer "tethered" at the computer station to the electronic health record (EHR) during a patient encounter.

- ***Workflow*** – Ochsner uses AI to speed up the prior authorization process with pharmacies.

These principles also apply for the most part to digital tools overall, with or without GenAI capabilities. Ultimately, patients should be fully informed and affirmatively consent to the use of digital tools.

F. Value-Based Care Organizations

VB-1. What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in the Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

As noted earlier, we are proud that our digital medicine offerings support and empower patients to manage their own health. Key factors that are stymying the scaling of our digital medicine programs are both the manner and amount that CMS pays for RPM. Current Medicare payment policy is incredibly restrictive and confusing with respect to when providers can use the associated billing codes. Medicare beneficiaries need access to both the device(s) and the data monitoring and disease management services from their providers. **Yet, CMS payment policy fails to cover 85% of the costs associated with the device and care – paying only 15% of what it costs Ochsner to provide these programs, which is not financially sustainable**.

Due to restrictions on what Medicare providers can offer for free to beneficiaries, providers generally cannot provide digital devices to patients at no cost and providers cannot waive the coinsurance for the benefit/service or the out-of-pocket cost for digital devices. This means beneficiaries in digital medicine and RPM programs can accrue a significant amount of cost-sharing and as such, many of them – as discussed earlier – choose not to enroll or disenroll due to cost. This financial barrier hinders access to care and limits the benefits of these programs to beneficiaries who can afford them. Digital medicine programs save money and improve outcomes – these are the types of services we want Medicare beneficiaries to utilize. Cost-sharing requirements exist to guard against over-consumption of care and services, yet in this case, we want to facilitate the utilization of these services and cost-sharing stands in the way of patient uptake.

<u>One option would be for CMS under the CMMI to allow for the following changes to be permitted for participants in the MSSP as there are built-in checks and balances related to quality, cost, and outcomes</u>:

- ***Create a Per Beneficiary Per Month Payment for RPM and Waive Cost-Sharing***: The current system of individual CPT codes is not efficient for providers or the Medicare program. A single, fixed amount paid per month (e.g., $55 for single disease program), like a subscription model, allows Medicare to pay for a service and permits providers to engage with beneficiaries and their data as frequently as necessary. Given the significant barrier that out-of-pocket costs pose to beneficiary participation and to ensure equitable access, cost-sharing for this secondary prevention service should be waived.

- ***Permit and Reimburse Multiple RPM Services Contemporaneously****:* At a minimum, Medicare should reimburse providers for multiple services performed contemporaneously for a single patient if a physician, or clinician under general supervision of the physician, recommends multiple RPM programs for the patient based on the patient's specific diagnoses. Recognizing there likely are economies of scale, the per beneficiary per month amount provided for each subsequent program would be prorated; a provider would not be paid three times the standard amount but rather a reduced proportion thereof for each subsequent program (e.g., $55 for single disease program, $75 for two disease programs).

- ***Another path forward would be for CMS to convene a group of stakeholders, including providers and beneficiaries, to discuss the benefits of RPM and further uncover the challenges to scaling these offerings and discuss alternative payment models that would reduce burdens on providers and decrease costs for beneficiaries***.

- **Another option would be for CMS, through CMMI, to work with providers, such as Ochsner, to design a pilot program or model that would utilize an alternative benefit design to facilitate access to RPM programs for a range of providers, not just MSSP participants**.

We respectfully request that CMS take action to improve its RPM payment policy so it supports – rather than impedes – the deployment of RPM technology for the management of multiple chronic conditions contemporaneously; doing so would advance the Trump Administration's goal of leveraging technology and innovation to improve chronic disease management, decrease costs, and increase access to care.

Conclusion

Ochsner thanks you again for issuing this RFI and for the opportunity to share lessons we have learned from deploying digital health technologies that expand and facilitate access to care, improve outcomes, and reduce costs. We stand ready to work with CMS and ASTP/ONC on "increasing beneficiary access to effective digital capabilities needed to make informed health decisions" and agree that the "digital ecosystem could ultimately expand access to personalized health guidance for patients to improve prevention and chronic disease management." Please feel free to contact Will Crump (William.crump@ochsner.org) should you have any questions or if we can be of assistance.

Sincerely,

Denise Basow, M.D.
Executive Vice President and Chief Digital Health Officer